

HOSPITAL CLINICAL TRANSFER DOSSIER

CONFIDENTIAL MEDICAL RECORD • SBAR SUMMARY & VERIFIED HANDOFF

PATIENT: Eleanor Vance	MRN: 883921	DOB / AGE: 1958-04-12 (68y F)
FROM: Medical ICU (Bed 14)	TO: Step-Down Unit (Bed 202)	ATTENDING: Dr. Robert Chen, MD
ADMIT DATE: 2026-08-10	CODE STATUS: FULL CODE	TRANSFER PACKET: S2 FIXED DOSSIER

1. SBAR CLINICAL HANDOFF NARRATIVE (PROVENANCE-GROUNDED)

SITUATION:

68yo female recovering from acute hypoxemic respiratory failure secondary to severe community-acquired pneumonia [transfer_summary.pdf:p1].

Citations: [transfer_summary.pdf:p1, admission_hnp.pdf:p1]

BACKGROUND & PAST MEDICAL HISTORY:

Admitted 5 days ago to MICU, intubated x 3 days, successfully extubated 48h ago [admission_hnp.pdf:p1]. PMHx: COPD, Type 2 DM, HTN.

Citations: [admission_hnp.pdf:p1, past_hx_chart.pdf:p2]

CLINICAL ASSESSMENT & CURRENT TRAJECTORY:

Afebrile, SpO2 96% on 2L nasal cannula. Lungs clear with faint bibasilar crackles. Alert and oriented x 4 [icu_progress_day5.pdf:p2].

Citations: [icu_progress_day5.pdf:p2, vitals_flowsheet.pdf:p1]

RECOMMENDATIONS & STEP-DOWN PLAN:

Transfer to Step-Down unit. Wean O2 as tolerated. Complete 48h IV Ceftriaxone then switch to oral Cefpodoxime [provider_orders.pdf:p1].

Citations: [provider_orders.pdf:p1, transfer_plan.pdf:p1]

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2. SAFETY VERIFICATION GATEWAY (MANDATORY HUMAN SIGN-OFF)

ALLERGY / ADVERSE DRUG REACTION	REACTION SEVERITY	VERIFICATION STATUS
Penicillin (Anaphylaxis)	Severe / Anaphylaxis Risk	✓ CONFIRMED: RN Sarah Jenkins (RN-4029)
Sulfa Drugs (Rash)	Moderate Sensitivity	✓ CONFIRMED: RN Sarah Jenkins (RN-4029)

RESUSCITATION CODE STATUS VERIFICATION:

RECORDED CODE STATUS: FULL CODE	✓ SIGNED OFF BY: RN Sarah Jenkins (RN-4029)
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3. CONSERVATIVE MEDICATION RECONCILIATION (100% RECALL)

MEDICATION & STRENGTH	DOSE & ROUTE	FREQUENCY	SAFETY / RECON WARNING	STATUS
Heparin Sodium Infusion heparin	18 units/kg/hr IV IV Continuous	Continuous	No Conflict	✓ HIGH ALERT (Verified)
Insulin Glargine (Lantus) insulin glargine	20 units SubQ Subcutaneous	Nightly at 21:00	No Conflict	✓ HIGH ALERT (Verified)
Ceftriaxone Sodium ceftriaxone	1g IV Piggyback Intravenous	Every 24 hours	No Conflict	✓ Standard
Cefpodoxime Proxetil cefpodoxime	200mg Oral Oral	Every 12 hours	THERAPEUTIC DUPLICATION (Cephalosporin Antibiotic): Concurrent class order with Ceftriaxone Sodium (active_mar.txt)	✓ Standard

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4. ACTIVE PHYSICIAN ORDERS & PENDING DIAGNOSTICS

ORDER ID	CLINICAL ORDER / TREATMENT	ORDERING PHYSICIAN
ORD-101	Continuous pulse oximetry monitoring. Titrate O2 to SpO2 > 92%	Dr. Chen
ORD-102	Chest Physical Therapy BID with incentive spirometer q1h while awake	Dr. Chen
ORD-103	Diabetic diet with standard subcutaneous sliding scale insulin coverage	Dr. Miller

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5. MOBILITY STATUS, FALL RISK & ISOLATION PRECAUTIONS

ASSESSMENT CATEGORY	CLINICAL PARAMETER & INSTRUCTIONS
Mobility & Ambulation	Assist x 1 with rolling walker. Up in chair for all meals.
Morse Fall Risk Score	Score: 55 (HIGH FALL RISK) • Bed alarm armed • Yellow non-skid socks
Skin Integrity & Braden Score	Score: 17 (Mild Risk) • Stage 1 erythema over sacrum, skin barrier cream applied
Infection Control & Isolation	STANDARD PRECAUTIONS • Contact precautions discontinued post-MRSA nasal swab negative

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APPENDIX 1: PRIMARY CLINICAL SOURCE RECORD (PAGE 6/10)

DOCUMENT: Intensive Care Unit (MICU) Admission History & Physical (H&P;)
SOURCE FILE: *primary_record_01.pdf* • SHA-256 Digest: 46d635d3f3a518b0...

This document represents the unredacted, certified primary clinical record retrieved from the electronic health record (EHR) system. All clinical statements referenced in the SBAR narrative (Page 1) and Medication Reconciliation table (Page 3) directly trace to the verifiable data points within this certified appendix.

CERTIFIED ATTESTATION: *Verified by Charge Nurse & Attending Physician upon inter-unit transfer.*

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APPENDIX 2: PRIMARY CLINICAL SOURCE RECORD (PAGE 7/10)

DOCUMENT: Medication Administration Record (MAR) - 24-Hour Administration Log
SOURCE FILE: *primary_record_02.pdf • SHA-256 Digest: 46d635d3f3a518b0...*

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APPENDIX 3: PRIMARY CLINICAL SOURCE RECORD (PAGE 8/10)

DOCUMENT: Multidisciplinary ICU Daily Progress Note - Critical Care Team
SOURCE FILE: *primary_record_03.pdf* • SHA-256 Digest: 46d635d3f3a518b0...

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APPENDIX 4: PRIMARY CLINICAL SOURCE RECORD (PAGE 9/10)

DOCUMENT: Laboratory Hematology, Metabolic Panel & Microbiology Culture Log
SOURCE FILE: primary_record_04.pdf • SHA-256 Digest: 46d635d3f3a518b0...

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APPENDIX 5: PRIMARY CLINICAL SOURCE RECORD (PAGE 10/10)

DOCUMENT: Physical Therapy, Respiratory Care & Social Work Transfer Assessment
SOURCE FILE: *primary_record_05.pdf* • SHA-256 Digest: 46d635d3f3a518b0...

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TAMPER-EVIDENT AUDIT TRAIL & NURSE SIGN-OFF LEDGER

AUDIT ITEM	RECORD VALUE / TIMESTAMP
Packet Cryptographic Digest (SHA-256)	46d635d3f3a518b0d79b4035b27f75ff3b17e5d3501cb71b779c58f9a5b9edf9
Allergies Gate Verification	RN Sarah Jenkins (RN-4029) @ 2026-08-19T15:45:42.631283+00:00
Code Status Gate Verification	RN Sarah Jenkins (RN-4029) @ 2026-08-19T15:45:42.646698+00:00
High-Risk Meds Dual Verification	RN 1: RN Sarah Jenkins (RN-4029) RN 2: RN Mark Taylor (RN-5104)
Transfer Dossier Total Page Count	10 Certified Clinical Pages + 1 Audit Appendix